

HOUSE No. 1227**The Commonwealth of Massachusetts**

PRESENTED BY:

Meghan K. Kilcoyne

To the Honorable Senate and House of Representatives of the Commonwealth of Massachusetts in General Court assembled:

The undersigned legislators and/or citizens respectfully petition for the adoption of the accompanying bill:

An Act relative to patient access to biomarker testing to provide appropriate therapy.

PETITION OF:

NAME:	DISTRICT/ADDRESS:	DATE ADDED:
<i>Meghan K. Kilcoyne</i>	<i>12th Worcester</i>	<i>1/16/2025</i>
<i>Michael P. Kushmerek</i>	<i>3rd Worcester</i>	<i>3/28/2025</i>
<i>Sean Reid</i>	<i>11th Essex</i>	<i>2/5/2025</i>
<i>Lindsay N. Sabadosa</i>	<i>1st Hampshire</i>	<i>2/5/2025</i>
<i>Natalie M. Higgins</i>	<i>4th Worcester</i>	<i>2/5/2025</i>
<i>Hannah Kane</i>	<i>11th Worcester</i>	<i>2/5/2025</i>
<i>Carole A. Fiola</i>	<i>6th Bristol</i>	<i>2/5/2025</i>
<i>Brian W. Murray</i>	<i>10th Worcester</i>	<i>2/5/2025</i>
<i>Jessica Ann Giannino</i>	<i>16th Suffolk</i>	<i>2/5/2025</i>
<i>Edward R. Philips</i>	<i>8th Norfolk</i>	<i>2/5/2025</i>
<i>Paul J. Donato</i>	<i>35th Middlesex</i>	<i>2/5/2025</i>
<i>David Paul Linsky</i>	<i>5th Middlesex</i>	<i>2/5/2025</i>
<i>Bradley H. Jones, Jr.</i>	<i>20th Middlesex</i>	<i>2/5/2025</i>
<i>Thomas M. Stanley</i>	<i>9th Middlesex</i>	<i>2/5/2025</i>
<i>Samantha Montaño</i>	<i>15th Suffolk</i>	<i>2/5/2025</i>
<i>John Francis Moran</i>	<i>9th Suffolk</i>	<i>2/5/2025</i>
<i>Rodney M. Elliott</i>	<i>16th Middlesex</i>	<i>2/5/2025</i>
<i>Steven Ultrino</i>	<i>33rd Middlesex</i>	<i>2/5/2025</i>

<i>Michael D. Brady</i>	<i>Second Plymouth and Norfolk</i>	<i>2/13/2025</i>
<i>Jacob R. Oliveira</i>	<i>Hampden, Hampshire and Worcester</i>	<i>2/13/2025</i>
<i>Natalie M. Blais</i>	<i>1st Franklin</i>	<i>2/13/2025</i>
<i>William F. MacGregor</i>	<i>10th Suffolk</i>	<i>2/13/2025</i>
<i>Daniel Cahill</i>	<i>10th Essex</i>	<i>2/13/2025</i>
<i>James C. Arena-DeRosa</i>	<i>8th Middlesex</i>	<i>2/13/2025</i>
<i>Angelo J. Puppolo, Jr.</i>	<i>12th Hampden</i>	<i>2/13/2025</i>
<i>Tram T. Nguyen</i>	<i>18th Essex</i>	<i>2/26/2025</i>
<i>Erika Uyterhoeven</i>	<i>27th Middlesex</i>	<i>2/26/2025</i>
<i>Rob Consalvo</i>	<i>14th Suffolk</i>	<i>2/26/2025</i>
<i>James Arciero</i>	<i>2nd Middlesex</i>	<i>2/26/2025</i>
<i>Paul McMurtry</i>	<i>11th Norfolk</i>	<i>2/26/2025</i>
<i>Paul K. Frost</i>	<i>7th Worcester</i>	<i>2/26/2025</i>
<i>Kevin G. Honan</i>	<i>17th Suffolk</i>	<i>2/26/2025</i>
<i>Ann-Margaret Ferrante</i>	<i>5th Essex</i>	<i>2/26/2025</i>
<i>Bruce E. Tarr</i>	<i>First Essex and Middlesex</i>	<i>3/14/2025</i>
<i>Steven Owens</i>	<i>29th Middlesex</i>	<i>3/14/2025</i>
<i>Mike Connolly</i>	<i>26th Middlesex</i>	<i>3/14/2025</i>
<i>Kimberly N. Ferguson</i>	<i>1st Worcester</i>	<i>3/14/2025</i>
<i>Patrick M. O'Connor</i>	<i>First Plymouth and Norfolk</i>	<i>3/14/2025</i>
<i>Kathleen R. LaNatra</i>	<i>12th Plymouth</i>	<i>3/14/2025</i>
<i>Richard G. Wells, Jr.</i>	<i>7th Norfolk</i>	<i>3/14/2025</i>
<i>Mark J. Cusack</i>	<i>5th Norfolk</i>	<i>3/14/2025</i>
<i>Adrian C. Madaro</i>	<i>1st Suffolk</i>	<i>3/14/2025</i>
<i>Ryan C. Fattman</i>	<i>Worcester and Hampden</i>	<i>3/14/2025</i>
<i>Richard M. Haggerty</i>	<i>30th Middlesex</i>	<i>3/14/2025</i>
<i>Joshua Tarsky</i>	<i>13th Norfolk</i>	<i>3/14/2025</i>
<i>Adrienne Pusateri Ramos</i>	<i>14th Essex</i>	<i>3/14/2025</i>
<i>Danillo A. Sena</i>	<i>37th Middlesex</i>	<i>3/14/2025</i>
<i>Amy Mah Sangiolo</i>	<i>11th Middlesex</i>	<i>4/7/2025</i>
<i>Kristin E. Kassner</i>	<i>2nd Essex</i>	<i>4/7/2025</i>
<i>Mindy Domb</i>	<i>3rd Hampshire</i>	<i>4/7/2025</i>
<i>Jeffrey Rosario Turco</i>	<i>19th Suffolk</i>	<i>4/7/2025</i>
<i>Vanna Howard</i>	<i>17th Middlesex</i>	<i>4/7/2025</i>
<i>Sean Garballey</i>	<i>23rd Middlesex</i>	<i>4/7/2025</i>
<i>Tara T. Hong</i>	<i>18th Middlesex</i>	<i>5/9/2025</i>
<i>Steven S. Howitt</i>	<i>4th Bristol</i>	<i>5/9/2025</i>
<i>Kate Donaghue</i>	<i>19th Worcester</i>	<i>5/9/2025</i>
<i>Marcus S. Vaughn</i>	<i>9th Norfolk</i>	<i>5/9/2025</i>

<i>Greg Schwartz</i>	<i>12th Middlesex</i>	<i>5/30/2025</i>
<i>Marjorie C. Decker</i>	<i>25th Middlesex</i>	<i>5/30/2025</i>
<i>Margaret R. Scarsdale</i>	<i>1st Middlesex</i>	<i>6/13/2025</i>
<i>Christine P. Barber</i>	<i>34th Middlesex</i>	<i>6/13/2025</i>

HOUSE No. 1227

By Representative Kilcoyne of Clinton, a petition (accompanied by bill, House, No. 1227) of Meghan K. Kilcoyne and others relative to cancer patient access to biomarker testing to provide appropriate therapy. Financial Services.

The Commonwealth of Massachusetts

In the One Hundred and Ninety-Fourth General Court
(2025-2026)

An Act relative to patient access to biomarker testing to provide appropriate therapy.

Be it enacted by the Senate and House of Representatives in General Court assembled, and by the authority of the same, as follows:

1 SECTION 1. Chapter 32A of the General Laws is hereby amended by inserting after
2 section 17Q, the following section:-

3 Section 17R. (a) As used in this section, the following words shall have the following
4 meanings:

5 “Biomarker” means a characteristic that is objectively measured and evaluated as an
6 indicator of normal biological processes, pathogenic processes, or pharmacologic responses to a
7 specific therapeutic intervention, including known gene-drug interactions for medications being
8 considered for use or already being administered. Biomarkers include but are not limited to gene
9 mutations, characteristics of genes or protein expression.

10 “Biomarker testing” is the analysis of a patient’s tissue, blood, or other biospecimen for
11 the presence of a biomarker. Biomarker testing includes but is not limited to single-analyte tests,

multi-plex panel tests, protein expression, and whole exome, whole genome, and whole transcriptome sequencing.

“Consensus statements” as used here are statements developed by an independent, multidisciplinary panel of experts utilizing a transparent methodology and reporting structure and with a conflict of interest policy. These statements are aimed at specific clinical circumstances and base the statements on the best available evidence for the purpose of optimizing the outcomes of clinical care.

“Nationally recognized clinical practice guidelines” as used here are evidence-based clinical practice guidelines developed by independent organizations or medical professional societies utilizing a transparent methodology and reporting structure and with a conflict of interest policy. Clinical practice guidelines establish standards of care informed by a systematic review of evidence and an assessment of the benefits and risks of alternative care options and include recommendations intended to optimize patient care.

(b) The commission shall provide to any active or retired employee of the commonwealth who is insured under the group insurance commission coverage for biomarker testing as defined in this section, pursuant to criteria established under subsection (c).

(c) Biomarker testing must be covered for the purposes of diagnosis, treatment, appropriate management, or ongoing monitoring of an enrollee’s disease or condition when the test is supported by medical and scientific evidence, including, but not limited to:

1. Labeled indications for an FDA-approved or -cleared test;
2. Indicated tests for an FDA-approved drug;

- 33 3. Warnings and precautions on FDA-approved drug labels;
- 34 4. Centers for Medicare and Medicaid Services (CMS) National Coverage
- 35 Determinations or any Medicare Administrative Contractor (MAC) Local Coverage
- 36 Determinations; or
- 37 5. Nationally recognized clinical practice guidelines and consensus statements.

38 (d) coverage as defined in subsection (c) of this section shall be provided in a manner that

39 limits disruptions in care including the need for multiple biopsies or biospecimen samples.

40 (e) In the case of coverage which requires prior authorization, a carrier or a utilization

41 review organization subject to this section must approve or deny a prior authorization request or

42 appeal and notify the enrollee, the enrollee's health care provider and any entity requesting

43 authorization of the service within 72 hours. If additional delay would result in significant risk

44 to the insured's health or well-being, a carrier or a utilization review organization shall approve

45 or deny the request within 24 hours. If a response by a carrier or utilization review organization

46 is not received within the time required under this paragraph, said request or appeal shall be

47 deemed granted.

48 (f) The patient and prescribing practitioner shall have access to a clear, readily accessible,

49 and convenient processes to request an exception to a coverage policy or an adverse utilization

50 review determination. The process shall be made readily accessible on the carrier's website.

51 SECTION 2. Chapter 118E of the General Laws is hereby amended by inserting after

52 section 10L, the following section:-

Section 10M. (a) As used in this section, the following words shall have the following meanings:

“Biomarker” means a characteristic that is objectively measured and evaluated as an indicator of normal biological processes, pathogenic processes, or pharmacologic responses to a specific therapeutic intervention, including known gene-drug interactions for medications being considered for use or already being administered. Biomarkers include but are not limited to gene mutations, characteristics of genes or protein expression.

“Biomarker testing” is the analysis of a patient’s tissue, blood, or other biospecimen for the presence of a biomarker. Biomarker testing includes but is not limited to single-analyte tests, multi-plex panel tests, protein expression, and whole exome, whole genome, and whole transcriptome sequencing.

“Consensus statements” as used here are statements developed by an independent, multidisciplinary panel of experts utilizing a transparent methodology and reporting structure and with a conflict of interest policy. These statements are aimed at specific clinical circumstances and base the statements on the best available evidence for the purpose of optimizing the outcomes of clinical care.

“Nationally recognized clinical practice guidelines” as used here are evidence-based clinical practice guidelines developed by independent organizations or medical professional societies utilizing a transparent methodology and reporting structure and with a conflict of interest policy. Clinical practice guidelines establish standards of care informed by a systematic review of evidence and an assessment of the benefits and risks of alternative care options and include recommendations intended to optimize patient care.

(b) The division and its contracted health insurers, health plans, health maintenance organizations, behavioral health management firms and third-party administrators under contract to a Medicaid managed care organization or primary care clinician plan shall provide coverage for biomarker testing as defined in this section, pursuant to criteria established under subsection (c).

(c) Biomarker testing must be covered for the purposes of diagnosis, treatment, appropriate management, or ongoing monitoring of an enrollee's disease or condition when the test is supported by medical and scientific evidence, including, but not limited to:

1. Labeled indications for an FDA-approved or -cleared test
2. Indicated tests for an FDA-approved drug;
3. Warnings and precautions on FDA-approved drug labels;
4. Centers for Medicare and Medicaid Services (CMS) National Coverage Determinations or any Medicare Administrative Contractor (MAC) Local Coverage Determinations; or
5. Nationally recognized clinical practice guidelines and consensus statements.

(d) coverage as defined in subsection (c) of this section shall be provided in a manner that limits disruptions in care including the need for multiple biopsies or biospecimen samples.

(e) In the case of coverage which requires prior authorization, a carrier or a utilization review organization subject to this section must approve or deny a prior authorization request or appeal and notify the enrollee, the enrollee's health care provider and any entity requesting authorization of the service within 72 hours. If additional delay would result in significant risk

to the insured's health or well-being, a carrier or a utilization review organization shall approve or deny the request within 24 hours. If a response by a carrier or utilization review organization is not received within the time required under this paragraph, said request or appeal shall be deemed granted.

(f) The patient and prescribing practitioner shall have access to a clear, readily accessible, and convenient processes to request an exception to a coverage policy or an adverse utilization review determination. The process shall be made readily accessible on the carrier's website.

SECTION 3. Chapter 175 of the General Laws is hereby amended by inserting after section 47KK, the following section:-

Section 47LL. (a) As used in this section, the following words shall have the following meanings:

"Biomarker" means a characteristic that is objectively measured and evaluated as an indicator of normal biological processes, pathogenic processes, or pharmacologic responses to a specific therapeutic intervention, including known gene-drug interactions for medications being considered for use or already being administered. Biomarkers include but are not limited to gene mutations, characteristics of genes or protein expression.

"Biomarker testing" is the analysis of a patient's tissue, blood, or other biospecimen for the presence of a biomarker. Biomarker testing includes but is not limited to single-analyte tests, multi-plex panel tests, protein expression, and whole exome, whole genome, and whole transcriptome sequencing.

“Consensus statements” as used here are statements developed by an independent, multidisciplinary panel of experts utilizing a transparent methodology and reporting structure and with a conflict of interest policy. These statements are aimed at specific clinical circumstances and base the statements on the best available evidence for the purpose of optimizing the outcomes of clinical care.

“Nationally recognized clinical practice guidelines” as used here are evidence-based clinical practice guidelines developed by independent organizations or medical professional societies utilizing a transparent methodology and reporting structure and with a conflict of interest policy. Clinical practice guidelines establish standards of care informed by a systematic review of evidence and an assessment of the benefits and risks of alternative care options and include recommendations intended to optimize patient care.

(b) An individual policy of accident and sickness insurance issued under section 108 that provides benefits for hospital expenses and surgical expenses and any group blanket policy of accident and sickness insurance issued under section 110 that provides benefits for hospital expenses and surgical expenses delivered, issued or renewed by agreement between the insurer and the policyholder, within or outside the commonwealth, shall provide benefits for residents of the commonwealth and all group members having a principal place of employment in the commonwealth for biomarker testing as defined in this section, pursuant to criteria established under subsection (c).

(c) Biomarker testing must be covered for the purposes of diagnosis, treatment, appropriate management, or ongoing monitoring of an enrollee’s disease or condition when the test is supported by medical and scientific evidence, including, but not limited to:

1. Labeled indications for an FDA-approved or -cleared test
 2. Indicated tests for an FDA-approved drug;
 3. Warnings and precautions on FDA-approved drug labels;
 4. Centers for Medicare and Medicaid Services (CMS) National Coverage Determinations or any Medicare Administrative Contractor (MAC) Local Coverage Determinations; or
 5. Nationally recognized clinical practice guidelines and consensus statements.
- (d) coverage as defined in subsection (c) of this section shall be provided in a manner that limits disruptions in care including the need for multiple biopsies or biospecimen samples.
- (e) In the case of coverage which requires prior authorization, a carrier or a utilization review organization subject to this section must approve or deny a prior authorization request or appeal and notify the enrollee, the enrollee's health care provider and any entity requesting authorization of the service within 72 hours. If additional delay would result in significant risk to the insured's health or well-being, a carrier or a utilization review organization shall approve or deny the request within 24 hours. If a response by a carrier or utilization review organization is not received within the time required under this paragraph, said request or appeal shall be deemed granted.
- (f) The patient and prescribing practitioner shall have access to a clear, readily accessible, and convenient processes to request an exception to a coverage policy or an adverse utilization review determination. The process shall be made readily accessible on the carrier's website.

SECTION 4. Chapter 176A of the General Laws is hereby amended by inserting after section 8MM, the following section:-

Section 8NN. (a) As used in this section, the following words shall have the following meanings:

“Biomarker” means a characteristic that is objectively measured and evaluated as an indicator of normal biological processes, pathogenic processes, or pharmacologic responses to a specific therapeutic intervention, including known gene-drug interactions for medications being considered for use or already being administered. Biomarkers include but are not limited to gene mutations, characteristics of genes or protein expression.

“Biomarker testing” is the analysis of a patient’s tissue, blood, or other biospecimen for the presence of a biomarker. Biomarker testing includes but is not limited to single-analyte tests, multi-plex panel tests, protein expression, and whole exome, whole genome, and whole transcriptome, sequencing.

“Consensus statements” as used here are statements developed by an independent, multidisciplinary panel of experts utilizing a transparent methodology and reporting structure and with a conflict of interest policy. These statements are aimed at specific clinical circumstances and base the statements on the best available evidence for the purpose of optimizing the outcomes of clinical care.

“Nationally recognized clinical practice guidelines” as used here are evidence-based clinical practice guidelines developed by independent organizations or medical professional societies utilizing a transparent methodology and reporting structure and with a conflict of interest policy. Clinical practice guidelines establish standards of care informed by a systematic

review of evidence and an assessment of the benefits and risks of alternative care options and include recommendations intended to optimize patient care.

(b) Any contract between a subscriber and the corporation under an individual or group hospital service plan that is delivered, issued or renewed within the commonwealth shall provide coverage for biomarker testing as defined in this section, pursuant to criteria established under subsection (c).

(c) Biomarker testing must be covered for the purposes of diagnosis, treatment, appropriate management, or ongoing monitoring of an enrollee's disease or condition when the test is supported by medical and scientific evidence, including, but not limited to:

1. Labeled indications for an FDA-approved or -cleared test
2. Indicated tests for an FDA-approved drug;
3. Warnings and precautions on FDA-approved drug labels;
4. Centers for Medicare and Medicaid Services (CMS) National Coverage Determinations or any Medicare Administrative Contractor (MAC) Local Coverage Determinations; or
5. Nationally recognized clinical practice guidelines and consensus statements.

(d) coverage as defined in subsection (c) of this section shall be provided in a manner that limits disruptions in care including the need for multiple biopsies or biospecimen samples.

(e) In the case of coverage which requires prior authorization, a carrier or a utilization review organization subject to this section must approve or deny a prior authorization request or

200 appeal and notify the enrollee, the enrollee's health care provider and any entity requesting
201 authorization of the service within 72 hours. If additional delay would result in significant risk
202 to the insured's health or well-being, a carrier or a utilization review organization shall approve
203 or deny the request within 24 hours. If a response by a carrier or utilization review organization
204 is not received within the time required under this paragraph, said request or appeal shall be
205 deemed granted.

206 (f) The patient and prescribing practitioner shall have access to a clear, readily accessible,
207 and convenient processes to request an exception to a coverage policy or an adverse utilization
208 review determination. The process shall be made readily accessible on the carrier's website.

209 SECTION 5. Chapter 176B of the General Laws is hereby amended by inserting after
210 section 4MM, the following section:-

211 Section 4NN. (a) As used in this section, the following words shall have the following
212 meanings:

213 "Biomarker" means a characteristic that is objectively measured and evaluated as an
214 indicator of normal biological processes, pathogenic processes, or pharmacologic responses to a
215 specific therapeutic intervention, including known gene-drug interactions for medications being
216 considered for use or already being administered. Biomarkers include but are not limited to gene
217 mutations, characteristics of genes or protein expression.

218 "Biomarker testing" is the analysis of a patient's tissue, blood, or other biospecimen for
219 the presence of a biomarker. Biomarker testing includes but is not limited to single-analyte tests,
220 multi-plex panel tests, protein expression, and whole exome, whole genome, and whole
221 transcriptome sequencing.

“Consensus statements” as used here are statements developed by an independent, multidisciplinary panel of experts utilizing a transparent methodology and reporting structure and with a conflict of interest policy. These statements are aimed at specific clinical circumstances and base the statements on the best available evidence for the purpose of optimizing the outcomes of clinical care.

“Nationally recognized clinical practice guidelines” as used here are evidence-based clinical practice guidelines developed by independent organizations or medical professional societies utilizing a transparent methodology and reporting structure and with a conflict of interest policy. Clinical practice guidelines establish standards of care informed by a systematic review of evidence and an assessment of the benefits and risks of alternative care options and include recommendations intended to optimize patient care.

(b) Any subscription certificate under an individual or group medical service agreement delivered, issued or renewed within the commonwealth shall provide coverage for biomarker testing as defined in this section, pursuant to criteria established under subsection (c).

(c) Biomarker testing must be covered for the purposes of diagnosis, treatment, appropriate management, or ongoing monitoring of an enrollee’s disease or condition when the test is supported by medical and scientific evidence, including, but not limited to:

1. Labeled indications for an FDA-approved or -cleared test
2. Indicated tests for an FDA-approved drug;
3. Warnings and precautions on FDA-approved drug labels;

242 4. Centers for Medicare and Medicaid Services (CMS) National Coverage
243 Determinations or any Medicare Administrative Contractor (MAC) Local Coverage
244 Determinations; or

245 5. Nationally recognized clinical practice guidelines and consensus statements.

246 (d) coverage as defined in subsection (c) of this section shall be provided in a manner that
247 limits disruptions in care including the need for multiple biopsies or biospecimen samples.

248 (e) In the case of coverage which requires prior authorization, a carrier or a utilization
249 review organization subject to this section must approve or deny a prior authorization request or
250 appeal and notify the enrollee, the enrollee's health care provider and any entity requesting
251 authorization of the service within 72 hours. If additional delay would result in significant risk
252 to the insured's health or well-being, a carrier or a utilization review organization shall approve
253 or deny the request within 24 hours. If a response by a carrier or utilization review organization
254 is not received within the time required under this paragraph, said request or appeal shall be
255 deemed granted.

256 (f) The patient and prescribing practitioner shall have access to a clear, readily accessible,
257 and convenient processes to request an exception to a coverage policy or an adverse utilization
258 review determination. The process shall be made readily accessible on the carrier's website.

259 SECTION 6. Chapter 176G of the General Laws is hereby amended by inserting after
260 section 4EE, as so appearing, the following section:-

261 Section 4FF. (a) As used in this section, the following words shall have the following
262 meanings:

“Biomarker” means a characteristic that is objectively measured and evaluated as an indicator of normal biological processes, pathogenic processes, or pharmacologic responses to a specific therapeutic intervention, including known gene-drug interactions for medications being considered for use or already being administered. Biomarkers include but are not limited to gene mutations, characteristics of genes or protein expression.

“Biomarker testing” is the analysis of a patient’s tissue, blood, or other biospecimen for the presence of a biomarker. Biomarker testing includes but is not limited to single-analyte tests, multi-plex panel tests, protein expression, and whole exome, whole genome, and whole transcriptome sequencing.

“Consensus statements” as used here are statements developed by an independent, multidisciplinary panel of experts utilizing a transparent methodology and reporting structure and with a conflict of interest policy. These statements are aimed at specific clinical circumstances and base the statements on the best available evidence for the purpose of optimizing the outcomes of clinical care.

“Nationally recognized clinical practice guidelines” as used here are evidence-based clinical practice guidelines developed by independent organizations or medical professional societies utilizing a transparent methodology and reporting structure and with a conflict of interest policy. Clinical practice guidelines establish standards of care informed by a systematic review of evidence and an assessment of the benefits and risks of alternative care options and include recommendations intended to optimize patient care.

(b) Any individual or group health maintenance contract that is issued or renewed within or without the commonwealth shall provide coverage for biomarker testing as defined in this section, pursuant to criteria established under subsection (c).

(c) Biomarker testing must be covered for the purposes of diagnosis, treatment, appropriate management, or ongoing monitoring of an enrollee's disease or condition when the test is supported by medical and scientific evidence, including, but not limited to:

1. Labeled indications for an FDA-approved or -cleared test
2. Indicated tests for an FDA-approved drug;
3. Warnings and precautions on FDA-approved drug labels;
4. Centers for Medicare and Medicaid Services (CMS) National Coverage Determinations or any Medicare Administrative Contractor (MAC) Local Coverage Determinations; or
5. Nationally recognized clinical practice guidelines and consensus statements.

(d) coverage as defined in subsection (c) of this section shall be provided in a manner that limits disruptions in care including the need for multiple biopsies or biospecimen samples.

(e) In the case of coverage which requires prior authorization, a carrier or a utilization review organization subject to this section must approve or deny a prior authorization request or appeal and notify the enrollee, the enrollee's health care provider and any entity requesting authorization of the service within 72 hours. If additional delay would result in significant risk to the insured's health or well-being, a carrier or a utilization review organization shall approve or deny the request within 24 hours. If a response by a carrier or utilization review organization

304 is not received within the time required under this paragraph, said request or appeal shall be
305 deemed granted.

306 (f) The patient and prescribing practitioner shall have access to a clear, readily accessible,
307 and convenient processes to request an exception to a coverage policy or an adverse utilization
308 review determination. The process shall be made readily accessible on the carrier's website.